

Policy on Informed Decision-Making, Care Planning and Patient Education

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Patient Rights Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» explains proposed care, risks, benefits, alternatives, and expected outcomes to patients and families; involves them in preparing and modifying the care plan; keeps them informed of diagnostic results and any change in condition; and provides multi-disciplinary counselling where appropriate.

1. Purpose

This policy explains how «Hospital Name» educates patients and/or family members to make informed decisions and involves them in the care planning and delivery process: explaining proposed care and its risks, benefits and alternatives; involving them in the care plan; informing them of diagnostic results and condition changes; and providing multi-disciplinary counselling when appropriate.

The formal consent process for high-risk procedures is covered in the hospital's other policies.

2. Scope

This policy applies to attending doctors, treating physicians and their teams, and all patients and/or family members receiving care at «Hospital Name».

3. Policy standards

«Hospital Name»'s attending doctors explain the proposed care, its risks, benefits, alternatives, expected results and possible complications to the patient and/or family in a language they can understand, and involve them in preparing and modifying the care plan.

Patients and/or family members are informed about diagnostic results, the diagnosis, and any change in the patient's condition in a timely manner, and receive multi-disciplinary counselling when the situation calls for it.

4. Non-negotiable rules

- Do not proceed with proposed care without explaining it, including risks, benefits and alternatives, to the patient and/or family in a language they can understand.
- Do not withhold explanation of diagnostic test results and the diagnosis from the patient and/or family.
- Do not delay explaining a change in the patient's condition — improvement, deterioration or complications — once the change is known.
- Do not exclude the multi-disciplinary team from counselling in situations that call for it.

5. What we do

5.1 Explain the proposed care, risks, benefits and alternatives

The proposed care, including referral to internal and/or external services, is discussed by the attending doctor with the patient and/or family members, in a language they can understand. Expected outcomes of treatment are explained at periodic intervals by the treating physician or their team. Possible complications are clearly communicated.

5.2 Involve the patient and family in the care plan

The care plan is prepared and modified in consultation with the patient and/or family members. During preparation, the patient and/or family are explained the various treatment options, risks and benefits. Religious, cultural and spiritual views of the patient and/or family are considered during care delivery, within the limits of applicable statutory requirements.

5.3 Inform patients and family of diagnostic results

Results of all diagnostic tests are explained, at least in broad terms, to the patient and family members, along with their implication on progress and treatment.

5.4 Explain any change in the patient's condition

The patient and/or family members are explained about any change in the patient's condition — improvement, deterioration or complications — in a timely manner.

5.5 Provide multi-disciplinary counselling when appropriate

«Hospital Name» identifies situations requiring multi-disciplinary counselling — such as counselling for family members of critically ill patients, potential organ donors and/or their

family, and long-stay patients. Members of the multi-disciplinary team — doctors from different specialties, nurses and physiotherapists — take part in the counselling as relevant to the situation.

6. Stop-work authority

Not applicable. This standard has no stop-work authority. Gaps in explaining proposed care, care-plan consultation, or condition updates are addressed through the hospital's quality monitoring and CAPA process (Section 8), not through an immediate stop-work trigger. Consent requirements for high-risk procedures are covered in the hospital's other policies.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring patients and families are informed and involved in decision-making across all departments.
Patient Rights Officer	Owns implementation of this policy; monitors compliance with explanation and counselling requirements.
Attending/Treating Doctor	Explains proposed care, risks, benefits, alternatives and outcomes; involves the patient/family in care-plan preparation; explains diagnostic results and condition changes; documents these discussions.
Nursing Superintendent	Supports patient/family understanding through nursing communication and escalates unaddressed information gaps.
Quality Coordinator	Audits this policy; holds training records and staff acknowledgements.

8. Quality monitoring

Monitoring area	What is monitored
Explanation of proposed care	Clinical notes/consultation records showing explanation of risks, benefits and alternatives.
Care-plan involvement	Medical record notes of care-plan consultation with patient/family.
Diagnostic result communication	Clinical notes showing diagnostic results were communicated.
Condition-change communication	Clinical notes showing timely communication of condition changes.
Multi-disciplinary counselling	Written situation list; counselling records showing attendance and participation.

This policy shall be reviewed at least once every year, or earlier if NABH standards change.

9. Training and staff acknowledgement

All attending doctors, treating physicians and their teams shall be familiar with the informed decision-making, care-planning, and patient-education requirements of this policy, at induction and periodically.

I have read the Policy on Informed Decision-Making, Care Planning and Patient Education of «Hospital Name» and understand my responsibilities under it.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all clinical staff, the Patient Rights Officer, department heads, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OPD	Out-Patient Department

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means the objective element is starred and requires documented evidence.

Objective Element	Level	Traceability to this policy
PRE.3.a	CORE	Sections 3 and 5.1 address explanation of proposed care, risks, benefits, alternatives, expected results and complications.
PRE.3.b	Achievement	Sections 3 and 5.2 address patient/family involvement in care-plan preparation and modification.
PRE.3.c	Commitment	Sections 3 and 5.3 address explanation of diagnostic results and diagnosis.
PRE.3.d	Commitment	Sections 3 and 5.4 address timely explanation of condition changes.

PRE.3.e	Achievement	Sections 3 and 5.5 address multi-disciplinary counselling when appropriate.
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13. Required Records/Evidence Checklist

Explanation of proposed care — PRE.3.a (CORE)

- Clinical notes or consultation records showing explanation of proposed care, risks, benefits and alternatives was given.

Care-plan involvement — PRE.3.b (Achievement)

- Medical record notes documenting care-plan consultation with the patient and/or family.

Diagnostic result communication — PRE.3.c (Commitment)

- Clinical notes showing diagnostic results and their implications were communicated.

Condition-change communication — PRE.3.d (Commitment)

- Clinical notes showing timely communication of condition changes, with dates.

Multi-disciplinary counselling — PRE.3.e (Achievement)

- Written list of situations requiring multi-disciplinary counselling.
- Counselling records (attendees, date, what was discussed).

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PRE.3.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

This policy is intended for accreditation and internal governance purposes and is not a statement of, or substitute for, compliance with any specific statute; «Hospital Name» remains separately responsible for meeting all applicable statutory and regulatory requirements.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

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